

12–16 WEEKS

ADVANCED

MASS BUILDER

PEPTIDE STACK

ADVANCED STEROID CYCLE FOR BULKING WITH PEPTIDES 12–16 WEEK MASS PROTOCOL

Testosterone · Nandrolone · Dianabol · IGF-1 LR3 · GHRP-6 · CJC-1295 · Full PCT

Maximum lean mass accumulation · GH pulse amplification · Peptide synergy

ELITE FITNESS GUIDE SERIES

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■ **MEDICAL DISCLAIMER:** Educational and informational purposes only. Consult a licensed physician before beginning any hormonal protocol.

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1. MASS BUILDING PHILOSOPHY

This protocol is engineered to produce **maximum lean muscle mass** over 12–16 weeks by combining a powerful anabolic base with cutting-edge peptide technology that amplifies natural GH pulsatility. The philosophy is **synergistic overload** — each compound serves a distinct mechanism with minimal overlap.

■ A landmark study published in the *New England Journal of Medicine* demonstrated that testosterone + resistance training produced significantly more lean mass than either alone, confirming the synergistic relationship between androgens and mechanical load.

Metric	12-Week Result	16-Week Result
Lean Mass Gain	12–18 lbs	18–28 lbs
Strength Increase (Squat/Bench)	30–60 lbs	50–80 lbs
Body Fat Change	±2%	±2–4%
Recovery Speed	40–60% improvement	40–60% improvement
Peptide Synergy Benefit	Additional 10–20% lean mass vs AAS alone	Same

2. CORE COMPOUND STACK & MECHANISMS

Compound	Name	Dose	Frequency	Duration	Mechanism & Role
Testosterone Enanthate	Test E	500–750 mg	2 pins/wk	12–16 wks	Primary anabolic — stimulates AR, increases protein synthesis, IGF-1 production, and nitrogen retention. Enanthate ester = 7–8 day half-life; stable blood levels with twice-weekly dosing.
Nandrolone Decanoate	Deca	400–600 mg	Weekly	12–16 wks	19-nor androgen — superior nitrogen retention (higher than testosterone), collagen synthesis stimulation (joint healing), low androgenic side effects. Aromatizes mildly; elevates prolactin — requires Cabergoline.
Dianabol (Methandrostenolone)	Dbol	30–50 mg	Daily oral	Wks 1–6	Oral 17-alpha alkylated — rapid AR activation, dramatically increases glycogen uptake and nitrogen retention. Used as 'kick-start' while long-ester Test/Deca build up. Aromatizes heavily — AI mandatory.

3. 12–16 WEEK DOSAGE TIMELINE

The protocol divides into 3 phases. The **Kick-Start Phase (Wks 1–6)** uses Dbol to rapidly elevate anabolic state while long esters build up. The **Main Mass Phase (Wks 7–12)** achieves peak anabolic environment. The optional **Extension Phase (Wks 13–16)** is for athletes aiming for maximum gains.

Phase	Weeks	Test E/C (per wk)	Deca (per wk)	Oral Dbol + AI Protocol
Kick-Start	1–2	500mg (250mg Mon+Thu)	400mg (Mon)	Dbol 30mg/day · Arimidex 0.5mg EOD · Caber 0.25mg Tue+Fri
Kick-Start	3–4	500mg (250mg Mon+Thu)	400mg (Mon)	Dbol 40mg/day · same AI · strength rises rapidly
Kick-Start	5–6	750mg (375mg Mon+Thu)	600mg (Mon)	Dbol 50mg/day · same AI · monitor liver (TUDCA mandatory)
Main Mass	7–12	750mg (375mg Mon+Thu)	600mg (Mon)	Dbol stopped · Arimidex 0.5mg EOD · Caber 0.25mg Tue+Fri
Extension*	13–14	500mg (250mg Mon+Thu)	400mg (Mon)	Arimidex 0.25mg EOD · Caber 0.25mg Tue+Fri · taper phase
Extension*	15–16	500mg (250mg Mon+Thu)	— stopped	Arimidex 0.25mg EOD · Deca clearing · pre-PCT prep

Day-Wise Weekly Injection & Dosing Schedule:

Day	Test E/C	Deca	AI & Ancillary	Oral (Kick-Start Wks 1–6)
Monday	375mg (Main) / 250mg (Kick)	400–600mg	Arimidex 0.5mg	Dbol 15–25mg with meal
Tuesday	—	—	Cabergoline 0.25mg	Dbol 15–25mg with meal
Wednesday	—	—	—	Dbol 15–25mg with meal
Thursday	375mg (Main) / 250mg (Kick)	—	Arimidex 0.5mg	Dbol 15–25mg with meal
Friday	—	—	Cabergoline 0.25mg	Dbol 15–25mg with meal

Day	Test E/C	Deca	AI & Ancillary	Oral (Kick-Start Wks 1–6)
Saturday	—	—	—	Dbol 15–25mg with meal
Sunday	—	—	—	Dbol 15–25mg with meal

After Wks 1–6: Remove Dbol. Continue Test E/C + Deca with same AI protocol through the main mass phase.

■ 4. PEPTIDE PROTOCOL

Peptides work at a fundamentally different level than anabolic steroids — they modulate **endogenous GH release** from the pituitary, increase satellite cell proliferation, enhance nutrient partitioning, and accelerate recovery. Their use alongside AAS produces results that neither can achieve alone.

Peptide	Full Name	Dose	Timing	Protocol	Mechanism & Effect
IGF-1 LR3	Long R3 Insulin-Like Growth Factor-1	40–60 mcg post-workout	Training days only (5×/wk max)	4–6 wks on / 4 wks off	Binds IGF-1R with 3× greater affinity than native IGF-1; half-life ~20 hours vs 15 min for native IGF-1. Directly stimulates muscle cell hyperplasia (new cell creation) and hypertrophy simultaneously. Most powerful muscle-building peptide available.
GHRP-6	Growth Hormone Releasing Hexapeptide	100 mcg × 3/day	Fasted morning, pre-workout, pre-bed	Throughout cycle	Ghrelin receptor agonist — triggers acute GH pulse from pituitary. Each injection produces a natural GH spike within 30 min. Strong hunger stimulation (beneficial during bulk). Pair with CJC-1295 for amplified GH pulse (5–10× individual peptide effect).
CJC-1295 DAC	Modified GHRH Analog with Drug Affinity Complex	2 mg once weekly	SC injection any time	Wks 1–12	GHRH analog with extended half-life (8 days due to DAC modification). Elevates baseline GH by 2–10× over normal physiological levels. Acts as a 'GH tide' beneath which GHRP-6 creates larger individual spikes.

Practical Injection Protocol:

- Injection 1 (Morning, fasted): GHRP-6 100 mcg + CJC-1295 100 mcg (non-DAC, if using daily option) subcutaneously
- Injection 2 (Pre-workout, 60 min before): GHRP-6 100 mcg — amplifies intra-workout GH and IGF-1
- Injection 3 (Pre-bed, 2+ hrs after last meal): GHRP-6 100 mcg — capitalizes on sleep GH pulse
- IGF-1 LR3: 40–60 mcg IM (intramuscular) into the worked muscle, within 30 min post-training
- CJC-1295 DAC 2 mg SC once weekly — any consistent day

■■ **IMPORTANT:** IGF-1 LR3 causes transient hypoglycemia. Always eat 20–30g carbohydrates before injecting. Have glucose tablets nearby. Never inject fasted.

■ 5. OPTIONAL HGH ADDITION

Pharmaceutical-grade Human Growth Hormone can be added to the peptide stack for compounding benefits. HGH works via the same pituitary/IGF-1 axis but provides **exogenous GH** rather than stimulating endogenous production.

HGH Quality	Source	Dose Range	Effect on IGF-1	Notes
Pharmaceutical Grade	Prescription only	2–4 IU/day	IGF-1 rises 100–300 ng/mL	Gold standard — exact concentration; expensive
Generic (176aa)	Research peptide suppliers	4–6 IU/day	Variable	Purity varies — test with kit before use
Fragment 176-191	Peptide supplier	500 mcg/day	Minimal IGF-1 effect	Fat loss only; no anabolic benefit

- Optimal timing: 2 IU fasted morning + 2 IU early afternoon (avoids nighttime hypoglycemia)
- Monitor fasting glucose and HbA1c — HGH causes insulin resistance; kits should include extra insulin sensitivity supplements
- IGF-1 blood test target on HGH: 250–350 ng/mL — above 400 ng/mL increases acromegaly risk with long-term use

■ 6. AI & PROLACTIN MANAGEMENT

Bulking protocols present unique estrogen management challenges. **Some estrogen elevation is actually desirable** during a mass phase — estrogen contributes to IGF-1 production, glycogen storage, and synovial joint fluid production. The target E2 for bulking is higher (35–50 pg/mL) than cutting (20–30 pg/mL).

■ *Research shows estradiol >50 pg/mL correlates with increased water retention and gynecomastia risk, while estradiol <15 pg/mL during a mass phase impairs IGF-1 signaling and reduces glycogen storage capacity — both counterproductive to muscle growth.*

- **Arimidex:** 0.5 mg twice weekly (Mon/Thu) — start conservative; adjust based on E2 blood work
- **Cabergoline:** 0.25 mg twice weekly — NON-NEGOTIABLE with Deca. Nandrolone activates progesterone and prolactin receptors; prolactin-induced gyno does not respond to Nolvadex
- Signs you need more AI: water weight, puffy nipples, high BP, lethargy, emotional blunting
- Signs you need less AI: joint cracking, low libido, depression, difficulty maintaining erection, poor pumps

7. POST CYCLE THERAPY (PCT)

■■ **IMPORTANT:** Deca-Durabolin has a half-life of approximately 15 days. PCT timing is critical: begin PCT 3–4 WEEKS after last Deca injection, and 2 weeks after last Test E/C injection. Starting PCT too early while Nandrolone is still active will suppress HPT axis recovery.

Compound	Dose	Timeline	Purpose & Mechanism
HCG	1000 IU	Every 3 days for 3 wks pre-PCT	LH analog — directly stimulates Leydig cell testosterone production. Prevents permanent testicular desensitization. Run before SERM therapy begins.
Clomid	100 mg/day wk1, 50 mg/day wks2–4	4 weeks	SERM — blocks ER at pituitary/hypothalamus, removes negative feedback, dramatically increases GnRH → LH → FSH cascade. Most powerful SERM for HPT axis restart.
Nolvadex	40 mg/day wk1, 20 mg/day wks2–6	6 weeks	SERM — anti-estrogenic at breast tissue + pituitary. Lower suppressive risk than Clomid for mood. Preferred solo SERM for milder cases; combined with Clomid for heavy cycles.
Vitamin E (Tocopherol)	400 IU/day	Throughout PCT	Antioxidant — protects Leydig cells from oxidative damage during hormonal fluctuation; improves fertility markers during PCT

8. MASS-PHASE NUTRITION SCIENCE

Nutrition is the foundation of mass building — anabolic compounds amplify your body's response to food, but they cannot create muscle from nothing. The mass-phase nutrition strategy optimizes **caloric surplus, protein synthesis windows, and insulin sensitivity**.

Caloric Surplus Calculation:

- Calculate TDEE: BMR (Mifflin-St Jeor) × Activity Factor (1.55 for moderate training)
- Mass phase surplus: +400–600 kcal above TDEE (lean bulk) or +600–1000 kcal (aggressive bulk)
- AAS reduces fat storage risk — larger surpluses are better tolerated than natural training
- Weigh weekly (same time, same conditions); adjust calories if gaining >1.5 lbs/week (excess fat)

Macro	Calculation	Example (200 lb Athlete)	Timing
Protein	1.8–2.2 g/lb body weight	360–440 g/day	Evenly distributed — every 3–4 hours
Carbohydrates	3–4 g/lb body weight	600–800 g/day	Majority around workouts; reduce at night
Fats	0.5–0.8 g/lb body weight	100–160 g/day	Morning and evening; avoid peri-workout
Total Calories	TDEE + 400–600	~3,800–4,200 kcal	Consistent daily intake

9. TRAINING FOR MAXIMUM HYPERTROPHY

On AAS, your **Maximum Recoverable Volume (MRV)** significantly increases — you can train harder, more frequently, and recover faster. The key is exploiting this recovery advantage without crossing into overtraining, which even AAS cannot fully prevent.

- **Frequency:** Each muscle group 2–3×/week. AAS increases protein synthesis duration from 24–36 hrs (natural) to 48+ hours
- **Volume:** 20–25 sets/muscle/week (natural MRV ~15–20 sets; AAS extends this)
- **Progressive overload:** Add weight, reps, or sets every session — non-negotiable for mass gain
- **Rep ranges:** 5–8 reps for compound movements (neurological strength); 10–15 for isolation (metabolic hypertrophy)
- **Sleep:** 8–9 hours minimum — 70% of GH release occurs during deep sleep. Non-negotiable for maximizing peptide and HGH benefits

10. ESSENTIAL SUPPLEMENT STACK

Supplement	Dose	Evidence & Purpose
TUDCA	500 mg/day	Liver protection for Dbol (oral 17-alpha alkylated compound); prevents cholestasis
Milk Thistle	600 mg/day	Sylmarin complex — hepatoprotective, anti-inflammatory for liver
Omega-3 (EPA+DHA)	5–6 g/day	Manages LDL/HDL ratio; anti-inflammatory; reduces AAS-induced dyslipidemia
CoQ10 (Ubiquinol)	300 mg/day	Mitochondrial support; cardiovascular protection; energy production
Creatine Monohydrate	5 g/day	Increases ATP production; 5–15% strength gain; improves high-rep performance on mass exercises
Beta-Alanine	3.2 g/day	Increases muscle carnosine; buffers lactic acid; enables extra reps in 8–15 rep range

Supplement	Dose	Evidence & Purpose
Whey + Casein Protein	50–80 g/day total	Whey post-workout (fast); casein pre-bed (slow) — maximizes 24hr protein synthesis
Zinc + Magnesium (ZMA)	Nightly	Improves sleep quality; supports testosterone production; cofactor for 300+ enzymatic reactions
Vitamin D3 + K2	5000 IU + 100 mcg	Testosterone production cofactor; muscle function; immune regulation
Glucosamine + Chondroitin	1500+1200 mg	Joint protection — heavy lifting + Deca on joints, but heavy loading still stresses cartilage

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